

When ESTROGEN Leaves the Building

AKA Your Body :)

Menopause, Parkinson's & Your Power
PPE Program | Boxing 4 Health



What Is Perimenopause & Menopause?

*It's not the end --
it's a new beginning.*

PERIMENOPAUSE

The lead-up to menopause -- usually starts in your late 30s to early 40s. Hormones begin to fluctuate erratically.

MENOPAUSE

Officially defined as 12 months after your last period. Average age is 51 -- but surgical menopause can happen much earlier.

WHY IT MATTERS FOR PD

This hormonal transition overlaps with the window when Parkinson's symptoms may first emerge or worsen. It's a critical time.

Estrogen & Your Brain

Why this matters for Parkinson's



Estrogen supports dopamine production -- the same system affected by Parkinson's



Acts as a neuroprotectant -- women develop PD later than men, and at lower rates



When estrogen drops, dopamine regulation becomes less stable

68%

of postmenopausal women
with Parkinson's report
worsening symptoms
after menopause

Kapelle et al., MDS Congress 2023

It's Not You -- It's Your Hormones

Symptoms that overlap between menopause and Parkinson's



Low Estrogen Symptoms

- Hot flashes
- Weight gain & bloating
- Vaginal dryness
- Brain fog
- Mood swings & anxiety
- Urinary incontinence



Parkinson's Symptoms That Worsen

- Fatigue (45% of women)
- Increased sweating (42%)
- Urinary issues (42%)
- Bradykinesia (23%)
- Tremor worsens (19%)
- Medication wearing off sooner



The Problem

- Symptoms look identical
- Doctors confuse them
- PD meds may need adjusting
- Only 1 in 21 women discussed this with a provider
- You may need both a neurologist AND a gynaecologist

Muscle Is Your Superpower

Building Strength in Midlife with Parkinson's

After menopause, lean muscle mass declines faster -- and for women with Parkinson's, this compounds existing challenges with balance, posture, and rigidity.

Muscle is an organ of longevity.

Strength training -- not cardio -- is the priority. Cardio is good. Lifting is essential.

In the PPE program, we build both.

01



Strength train 2-3x/week

Boosts metabolism, preserves muscle, improves PD motor function

02



Increase protein intake

Target 1.6g per kg body weight. Prioritize protein at every meal.

03



Stay hydrated

Estrogen loss causes dryness. Aim for 2L of water per day.

You're in the Driver's Seat

What to start doing today



Talk to Your Doctor

Share what you've learned today. Ask about adjusting your PD medications around hormonal changes. Advocate for yourself.



Lift Heavier Than You Think

Weight training is more important than cardio. Start where you are and progress. Your bones, muscles, and brain all benefit.



Explore HRT

Hormone Replacement Therapy may improve symptoms -- discuss with your gynaecologist AND neurologist together. One size does not fit all.



Track Your Symptoms

Note when symptoms change relative to hormonal shifts. This information is gold for your healthcare team.



Eat to Support Your Brain

Anti-inflammatory foods: berries, olive oil, fatty fish. Limit sugar -- inflammation worsens both menopause and PD. Protein first.

YOU GOT THIS.

*No one knows your body better than you.
Track your symptoms, explore your options, and advocate for what works.*



You are not alone in this.
The PPE program is built for you.



Estrogen leaving doesn't mean
your power leaves with it.



You train. You push. You adapt.
That is your superpower.